

PRETERM LABOUR

IDENTIFICATION & MANAGEMENT



IDENTIFY

Recognize symptoms, signs and assess risk



MANAGE

General measures, tocolysis, monitoring and timely transfer



MONITOR

Maternal and fetal monitoring based on gestational age and severity



ANTENATAL CORTICOSTEROID

Betamethasone or Dexamethasone to improve neonatal outcomes



REPEATED ANTENATAL CORTICOSTEROID

A single repeat course may be considered if risk of preterm birth persists

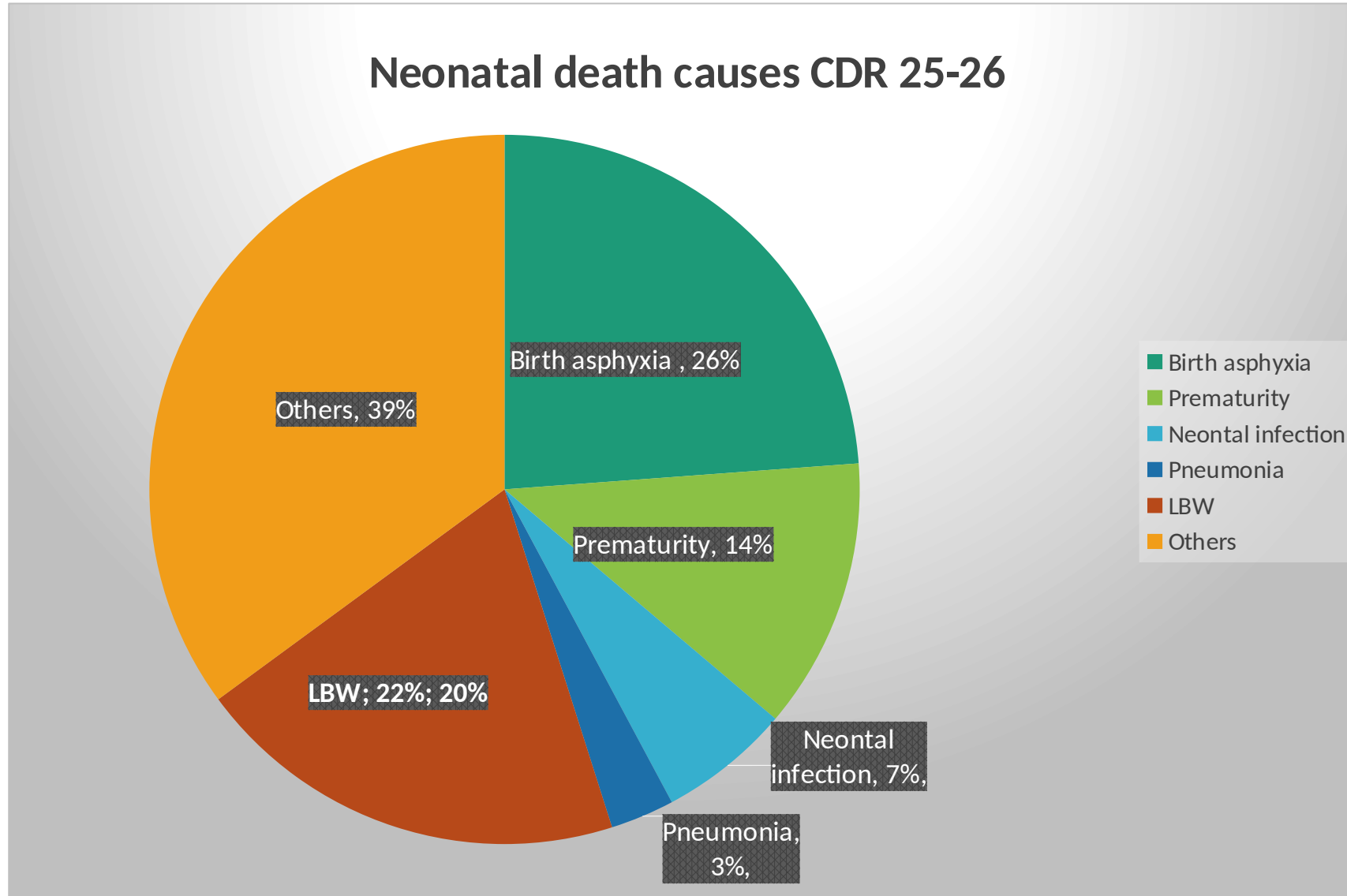


FOLLOW UP

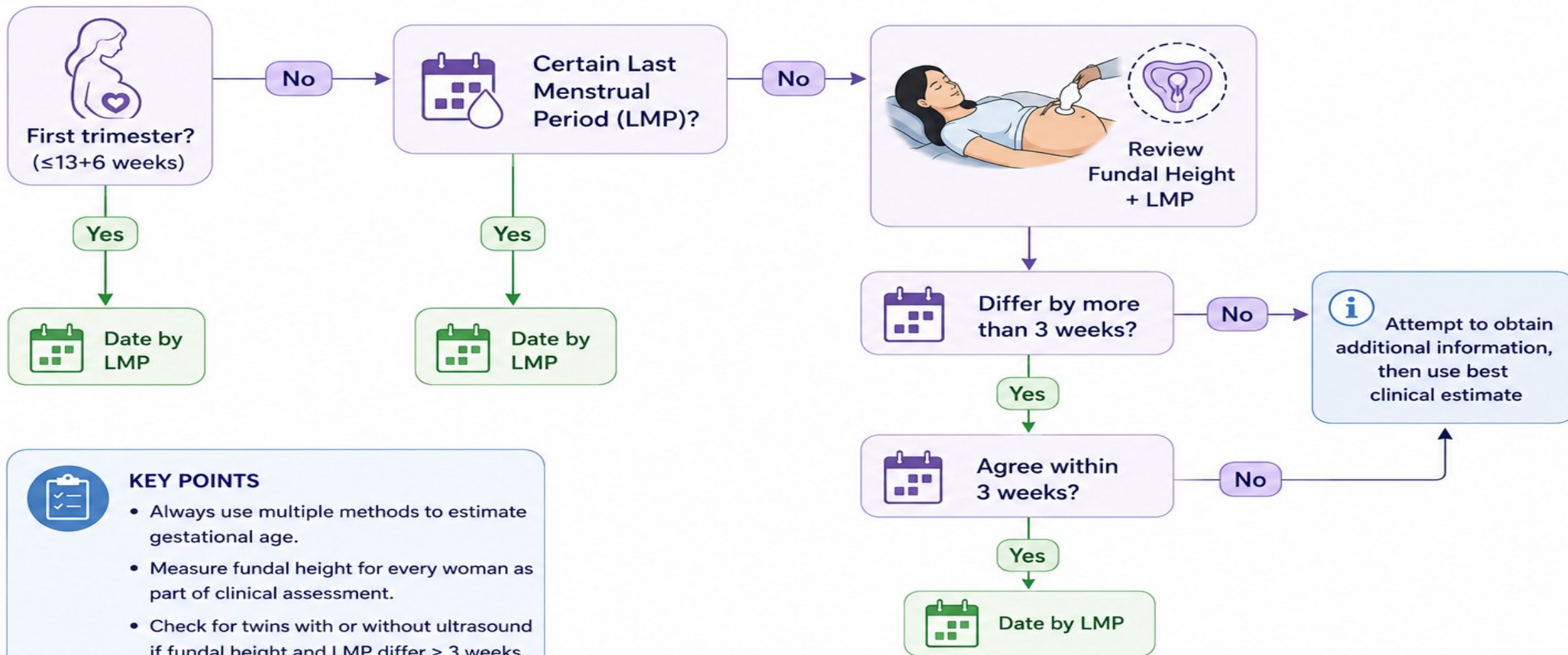
Reassess, counsel and plan ongoing care after stabilization

Topic 6

Preterm birth - A leading cause of neonatal deaths in India



HOW TO DATE PREGNANCY



KEY POINTS

- Always use multiple methods to estimate gestational age.
- Measure fundal height for every woman as part of clinical assessment.
- Check for twins with or without ultrasound if fundal height and LMP differ > 3 weeks.

→ Yes

→ No



Aim: Use the most accurate information available for the best clinical decision-making.

Recognizing true labor

True labor pain	False labor pain
■ Regular and predictable	■ Irregular
■ Felt first in lower back and sweeps towards lower abdomen	■ Remains confined to lower abdomen
■ Not relieved by rest	■ Often relieved by rest
■ Increase in duration, intensity and frequency with time	■ Does not increase in duration, intensity or frequency
■ “Show” blood stained mucus discharge present	■ “Show” absent
■ Accompanied by cervical changes	■ Not accompanied by cervical changes

Interventions done in mother to prevent neonatal complications:

- Antenatal Corticosteroids
- Antibiotics
- Tocolytics

- ANCS can be given by MPHWF at periphery with referral of case for delivery at higher centre (if delivery is not imminent)
- Decision of other interventions is taken by doctor only at higher center.

Antenatal corticosteroids

- Estimation of **correct gestational age** is a critical function before administering ANCS
- For true preterm labor between **24-34 weeks** gestation give antenatal corticosteroids to mother for **baby's lung maturity**
- **What ANCS to give?**

Injection Dexamethasone 6 mg intramuscularly, 12 hourly 4 doses

- If women is in initial stage of labor 1st dose of ANC should be administered by MPHW(F) or CHO and refer to higher center.
- Once 4 doses completed, no need to give another dose for rest of the pregnancy period.

When to give and when not to give ANCS

Indications

- True preterm labor (between 24-34 weeks of gestation)
- Conditions that lead to imminent delivery (between 24-34 weeks of gestation)
 - Antepartum hemorrhage
 - Preterm pre-labor rupture of membranes
 - Severe pre-eclampsia/eclampsia

Contraindications

- Frank chorioamnionitis (absolute contraindication)
- Signs and symptoms of chorioamnionitis
 - H/O fever, lower abdominal pain
 - Foul smelling vaginal discharge
 - Tender uterus
 - Maternal and fetal tachycardia

Key messages

- Correct estimation of gestational age to diagnose preterm delivery is critical for prevention of prematurity related complications
- Administer antenatal corticosteroid injection to the pregnant women between 24-34 weeks of gestation who are in true labor or have conditions that lead to imminent delivery
- Use Inj. Dexamethasone 6 mg intramuscularly repeated every 12 hours to complete a regimen of four doses
- Give one dose of Inj. Dexamethasone even if delivery is imminent or the woman is referred
- Repeated course of antenatal steroids is not recommended
- Be prepared for management of newborn resuscitation and warmth